INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Cobimetinib + vemurafenib — advanced melanoma

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
28 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00373
Version 3

Reference treatment cycle

Treatment stages

01 **Days 1–21: both medicines**
Three weeks

02 **Days 22–28: vemurafenib continues**
One week

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact the team promptly for new blurred or lost vision, eye pain, breathlessness or ankle swelling, palpitations or fainting, severe muscle pain or weakness with dark urine, unusual bleeding, severe diarrhoea, a blistering rash or yellow skin or eyes.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

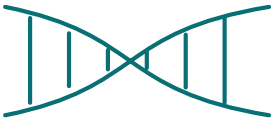
What each medicine does

To control advanced BRAF V600-mutated melanoma by blocking BRAF and MEK in the same growth pathway; the two medicines have different daily schedules.

MEK TARGETED TREATMENT

Cobimetinib

Cotellic



Blocks a second growth signal

Cobimetinib inhibits MEK, further along the growth-signalling pathway affected by BRAF. It is combined with vemurafenib. Cobimetinib has a seven-day planned break in each 28-day cycle; vemurafenib continues through that break unless the team holds it.

Blocks a second growth signal → Treatment plan and safety checks → Report new symptoms early

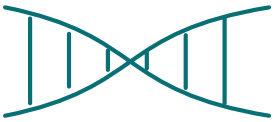
What to watch for

Report new vision changes, muscle pain with dark urine, breathlessness, ankle swelling or unusual bleeding.

BRAF TARGETED TREATMENT

Vemurafenib

Zelboraf



Blocks an altered BRAF signal

Vemurafenib blocks the altered BRAF V600 protein that can drive melanoma-cell growth. A confirmed tumour BRAF V600 result is needed. Healthy tissues can also be affected; tablets need the same careful monitoring as treatment given in hospital.

Blocks an altered BRAF signal → Treatment plan and safety checks → Report new symptoms early

What to watch for

Report new skin growths, sunburn-like reactions, eye pain or blurred vision, palpitations or fainting.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding your diagnosis and treatment goal

Use your own pathology, stage and agreed plan alongside this guide.

Your diagnosis and stage

Record where the melanoma began, its pathology result and the sites involved now. This pathway treats melanoma that cannot be fully removed or has spread. Treatment after complete removal has a different aim and a separate plan.

The tumour test

The laboratory checks a sample of the melanoma for a BRAF V600 change. The result can identify a target for certain medicines. A change found in the tumour does not, by itself, establish an inherited condition.

Scans and local treatments

Add planned scans, any operation and any radiotherapy to the same calendar. Ask what each part is intended to achieve and which team arranges it.

New changes still matter

Tell your team about a changing mole, a persistent sore, a new lump or an unexplained symptom. Improvement in a known tumour does not explain a new problem elsewhere.

Your medicines and overall plan

The schedule

Every 28 days: cobimetinib once daily on days 1–21, then seven days off; vemurafenib twice daily continues on all 28 days unless held by the team.

How treatment works

Cobimetinib inhibits MEK, further along the growth-signalling pathway affected by BRAF. It is combined with vemurafenib. Cobimetinib has a seven-day planned break in each 28-day cycle; vemurafenib continues through that break unless the team holds it. Vemurafenib blocks the altered BRAF V600 protein that can drive melanoma-cell growth. A confirmed tumour BRAF V600 result is needed. Healthy tissues can also be affected; tablets need the same careful monitoring as treatment given in hospital.

Important boundaries

Only cobimetinib has the planned days 22–28 break. Vemurafenib continues. electrocardiogram (ECG) measures rhythm; an echocardiogram or other left ventricular ejection fraction (heart pumping measurement) (LVEF) test measures pumping function, and one does not replace the other. Radiation treatment needs a separate coordinated plan.

Evidence and choices

Median progression-free survival: 12.3 versus 7.2 months (HR 0.58, 95% CI 0.46–0.72). Median overall survival: 22.3 versus 17.4 months (HR 0.70, 95% CI 0.55–0.90). These are group results from that study, not a prediction for you. Direct comparison was against vemurafenib alone. It does not rank this regimen against other BRAF/MEK pairs or immunotherapy.

Decisions, support and useful records

Your priorities belong in the treatment discussion.

Benefits and alternatives

Ask what the treatment aims to achieve, what alternatives are suitable, how likely important benefits are in your situation and how treatment burden or uncertainty affects that choice. A trial average cannot promise an individual outcome.

Support alongside treatment

Supportive care addresses symptoms and daily life during cancer treatment. Palliative care can be part of that support; accepting it does not mean you must stop anticancer treatment.

A person you trust

Choose whether to involve a relative or supporter and what may be shared with them. Ask for an interpreter or a different information format. Check that you can explain the next dose, tests and urgent-contact plan in your own words.

Your document set

Keep the current treatment summary, medicine list, test plan and contact card together. Use the printed instructions when you need them; the QR code is an optional link to further information.

What the treatment evidence can and cannot tell you

Evidence needs to match the medicine, dose combination, disease stage and previous treatment.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your treatment phases

Treatment stages

01 **Days 1–21: both medicines**
Three weeks

02 **Days 22–28: vemurafenib continues**
One week

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

THREE WEEKS

Days 1–21: both medicines

Cobimetinib once daily | **Vemurafenib twice daily**

Take both medicines according to their separate labels. Review symptoms, tests and dose changes before repeating.

ONE WEEK

Days 22–28: vemurafenib continues

Planned cobimetinib break | **Vemurafenib continues twice daily**

Do not stop vemurafenib just because cobimetinib has its planned break. A toxicity hold is a different instruction and must name which medicine is affected.

BASELINE → TREATMENT → FOLLOW-UP

Your testing and review plan

A reference cycle is not a personalised calendar. Tests listed as symptom-triggered or individual are not presented as mandatory tests at every visit. No universal imaging or tumour-marker interval is invented.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00373 v3: current regimen source · EMA Cotellic product information · EMA Zelboraf product information

Tests in your plan

Lactate dehydrogenase (LDH)

WHEN

Before treatment and before each 28-day cycle.

A blood result followed alongside the clinical assessment in this regimen.

A change is interpreted with other findings; it does not establish tumour growth by itself.

Creatine kinase (CK / CPK) and creatinine

WHEN

Before treatment and monthly during treatment, or sooner for symptoms.

Checks for muscle injury and its effect on the kidneys.

Report new muscle pain, weakness or dark urine promptly.

THE WHOLE TREATMENT JOURNEY

Cobimetinib + vemurafenib — advanced melanoma: treatment and checks

Advanced melanoma

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Safety tests and symptoms NCCP 00373	Confirm BRAF V600, blood count, kidney and liver function, LDH, and baseline CPK (muscle enzyme) and creatinine. Check ECG, LVEF (heart pumping function), medicines and skin; record vision symptoms.	Blood count, kidney/liver tests and LDH before each cycle. Current Cotellic product information adds CPK and creatinine monthly, and LVEF after the first month then at least every three months until treatment stops. ECG is every four weeks for the first 12 weeks, then every 12 weeks and after dose changes. Arrange monthly skin checks under current vemurafenib product information. New visual symptoms need prompt eye assessment; symptoms can require extra tests between scheduled checks.	Contact the team promptly for new blurred or lost vision, eye pain, breathlessness or ankle swelling, palpitations or fainting, severe muscle pain or weakness with dark urine, unusual bleeding, severe diarrhoea, a blistering rash or yellow skin or eyes.	After restarting reduced-dose cobimetinib, NCCP and product information specify LVEF at two, four, ten and sixteen weeks, then as indicated. Vemurafenib skin surveillance continues up to six months after stopping or until another anticancer therapy starts. Its other-cancer surveillance includes head-and-neck checks and chest computed tomography (CT), coordinated separately from melanoma-response imaging. A routine cobimetinib break is not evidence of toxicity recovery.	Do not restart held treatment until the team confirms the medicine, dose and timing. Safety tests and unresolved symptoms are reviewed first.
Scans and tumour results NCCP 00373	Keep the pathology, stage and relevant biomarker report.	Response scans follow the individual disease plan, separate from safety bloods.	Report new symptoms rather than waiting for a scan.	Ask which scan, its planned date and who will explain the result.	A treatment hold or stop may change the next scan or follow-up date.

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Written plan and contacts NCCP 00373	Record actual oncology and out-of-hours numbers; examples are not working contact details.	Bring your medicine list and record the dose actually given or taken.	Keep the wallet card and written urgent-symptom plan available.	Ask for a printed updated plan when treatment changes.	Keep the last-dose date and any steroid, hormone or other follow-up instructions.

What each check is for

1



Days 1–21: both medicines

Three weeks

To control advanced BRAF V600-mutated melanoma by blocking BRAF and MEK in the same growth pathway; the two medicines have different daily schedules.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

2

Days 22–28: vemurafenib continues

One week

To control advanced BRAF V600-mutated melanoma by blocking BRAF and MEK in the same growth pathway; the two medicines have different daily schedules.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Taking or receiving treatment

Usual starting doses are cobimetinib 60 mg once daily on days 1–21 and vemurafenib 960 mg twice daily, about 12 hours apart, continuously. Cobimetinib can be taken with or without food. Avoid regularly taking both daily vemurafenib doses on an empty stomach. Swallow both medicines whole and follow the separate labels.

STEP 2

Missed, vomited or held treatment

For cobimetinib, take a missed dose only if at least 12 hours remain before the next dose. For vemurafenib the interval is at least four hours. Otherwise skip the missed dose; do not double up. If you vomit after either medicine, do not take an extra dose. A hold or reduction may apply to one medicine or both: obtain a written component-specific restart plan.

STEP 3

Appointments and supply

Before each visit, check four things: the blood-test booking, how you will get there, your medicine supply and any help you need. Ask who will review the results and tell you whether the next dose can go ahead.

STEP 4

Paper copies and contacts

Keep the handbook, short guide and treatment card in one place. Ask your team to fill in the real daytime and emergency numbers and print a replacement when the plan changes. A phone is optional.

Taking or receiving treatment safely

Keep the written prescription and actual treatment record together.

Other medicines

Give the team a complete list of prescribed, over-the-counter and herbal medicines, supplements and allergies. Ask before adding a new medicine; treatment interactions and symptom-masking can matter.

Practical arrangements

Write down where blood samples are taken, who receives the results and how you obtain the next appointment or medicine supply. Request a printed plan if that is easier to use.

After a treatment hold, change or completion

The next step should be written down, not assumed.

Follow-up and late effects

After restarting reduced-dose cobimetinib, NCCP and product information specify LVEF at two, four, ten and sixteen weeks, then as indicated. Vemurafenib skin surveillance continues up to six months after stopping or until another anticancer therapy starts. Its other-cancer surveillance includes head-and-neck checks and chest CT, coordinated separately from melanoma-response imaging. A routine cobimetinib break is not evidence of toxicity recovery.

A treatment summary

Ask for a treatment summary showing what you received, important side effects, admissions, the last dose and the follow-up team. Share the current version with your GP and other clinicians.



Your next review

Before leaving, confirm the next test and review, how you will receive results and whom to call sooner. Keep those numbers available after treatment ends.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Daily tablets / capsules	 **Days 1–21** Every 28 days: cobimetinib once daily on days 1–21, then seven days off; vemurafenib twice daily continues on all 28 days unless held by the team.
Vemurafenib continues; cobimetinib break	 **Days 22–28** Continue vemurafenib twice daily; no cobimetinib on days 22–28 unless the team has issued a different written plan.
Safety and symptom review	 **Days 1–28** Blood count, kidney/liver tests and LDH before each cycle. Current Cotellic product information adds CPK and creatinine monthly, and LVEF after the first month then at least every three months until treatment stops. ECG is every four weeks for the first 12 weeks, then every 12 weeks and after dose changes. Arrange monthly skin checks under current vemurafenib product information. New visual symptoms need prompt eye assessment; symptoms can require extra tests between scheduled checks.

Schedule and review: Every 28 days: cobimetinib once daily on days 1–21, then seven days off; vemurafenib twice daily continues on all 28 days unless held by the team.

Holds, missed treatment and phase changes: For cobimetinib, take a missed dose only if at least 12 hours remain before the next dose. For vemurafenib the interval is at least four hours. Otherwise skip the missed dose; do not double up. If you vomit after either medicine, do not take an extra dose. A hold or reduction may apply to one medicine or both: obtain a written component-specific restart plan.

> **Why these bands stay broad:** This is a reference schedule. Your prescribed dose, blood results, symptoms and the team's written plan determine whether treatment goes ahead. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00373 v3: current regimen source ↗ · EMA Cotellic product information ↗ · EMA Zelboraf product information ↗

There are no guaranteed "safe days". The prescribed dose, treatment decision and local urgent-contact plan take priority. Call 112 or 999 for severe breathing difficulty, severe chest pain, collapse, a new seizure, sudden weakness or speech difficulty, or uncontrolled bleeding. Follow your oncology unit's written urgent-contact plan for other new or worsening symptoms; do not wait for the next appointment.

Between-treatment notes

Day 1–21: At home each day

Usual starting doses are cobimetinib 60 mg once daily on days 1–21 and vemurafenib 960 mg twice daily, about 12 hours apart, continuously. Cobimetinib can be taken with or without food. Avoid regularly taking both daily vemurafenib doses on an empty stomach. Swallow both medicines whole and follow the separate labels. For cobimetinib, take a missed dose only if at least 12 hours remain before the next dose. For vemurafenib the interval is at least four hours. Otherwise skip the missed dose; do not double up. If you vomit after either medicine, do not take an extra dose. A hold or reduction may apply to one medicine or both: obtain a written component-specific restart plan.

Day 22–28: Planned cobimetinib break

Vemurafenib continues twice daily. The seven-day break applies to cobimetinib only. Record any additional treatment hold separately and confirm the next cobimetinib start with the team.

Support around the medicines

1 Supportive medicines and symptom plan

NCCP lists each component as minimal-to-low emetogenic risk; follow local and individual anti-sickness policy. Agree a diarrhoea, fluid-intake and nausea plan; contact the team if symptoms persist or you cannot drink. Check all prescribed, over-the-counter and herbal medicines for interactions before taking them. The seven-day cobimetinib break is planned; extra breaks or restarting doses require the team's instructions.

2 Skin and daily care

Vemurafenib can make skin very sensitive to sunlight. Use UVA/UVB protection, protective clothing and sun protection factor (SPF) 30 or higher with lip protection. Continue skin checks for new growths or changing lesions. Do not assume combining the medicines removes the need for skin surveillance.

3 Fertility, pregnancy and breastfeeding

The cobimetinib source specifies two effective contraception methods during treatment and for at least three months afterwards; vemurafenib requires at least six months after its last dose. The combined plan therefore needs to cover the longer six-month vemurafenib interval and account for reduced hormonal-contraceptive effectiveness. Discuss fertility and breastfeeding before treatment.

Tests, results and treatment decisions

Safety tests and scans answer different questions.

Before treatment

Confirm BRAF V600, blood count, kidney and liver function, LDH, and baseline CPK (muscle enzyme) and creatinine. Check ECG, LVEF (heart pumping function), medicines and skin; record vision symptoms.

During treatment

Blood count, kidney/liver tests and LDH before each cycle. Current Cotellic product information adds CPK and creatinine monthly, and LVEF after the first month then at least every three months until treatment stops. ECG is every four weeks for the first 12 weeks, then every 12 weeks and after dose changes. Arrange monthly skin checks under current vemurafenib product information. New visual symptoms need prompt eye assessment; symptoms can require extra tests between scheduled checks.

What a result means

Ask which result is abnormal, whether the finding needs a repeat test and whether treatment is held, changed or stopped. Do not use a reference range or one normal result to decide independently that a dose is safe.

What to expect between visits

A quiet treatment day does not rule out later problems.

Supportive medicines

A useful daily record

Record symptoms, temperature when unwell, bowel changes, fluid intake when reduced, missed doses and any medicines used for relief. Note how symptoms affect eating, walking, sleep and normal activities; this helps the team judge severity.

When life is affected

Bring up difficulties with eating, sleep, pain, energy, work or caring for someone. The team can arrange symptom support while cancer treatment continues.

Skin, body changes and daily care

Use the detailed body map to locate symptoms, not to diagnose them.

The body map

The organ anchors show possible locations and warning symptoms. They do not show how common each effect is, and a symptom may have another cause. New symptoms should be assessed in their clinical context.

Food, fluids and activity

Choose food, fluids and movement that you can manage, within any personal restrictions. Ask for help early if you are losing weight, drinking less or finding everyday activity harder.

Keep checking the plan

A new medicine, fever, hospital admission, procedure or radiotherapy may change treatment timing. Tell the oncology team and take the wallet card or medicine list to other appointments.

Fertility, pregnancy and breastfeeding

Discuss these issues before the first dose whenever possible.

Fertility preservation

Ask whether treatment or the cancer may affect fertility and whether fertility preservation is appropriate before treatment. A change in periods or sexual function does not reliably establish infertility.

Written dates and methods

Record the agreed method, last treatment date and how long precautions continue. Different medicines have different intervals, so ask for clarification after a switch or combination change.

Sexual health

You can request a private discussion about intimacy, sexual symptoms or relationships. Ask for help if the agreed pregnancy precautions are difficult to follow.

Questions worth asking

Which tests are needed?

Confirm BRAF V600, blood count, kidney and liver function, LDH, and baseline CPK (muscle enzyme) and creatinine. Check ECG, LVEF (heart pumping function), medicines and skin; record vision symptoms. Blood count, kidney/liver tests and LDH before each cycle. Current Cotellic product information adds CPK and creatinine monthly, and LVEF after the first month then at least every three months until treatment stops. ECG is every four weeks for the first 12 weeks, then every 12 weeks and after dose changes. Arrange monthly skin checks under current vemurafenib product information. New visual symptoms need prompt eye assessment; symptoms can require extra tests between scheduled checks. After restarting reduced-dose cobimetinib, NCCP and product information specify LVEF at two, four, ten and sixteen weeks, then as indicated. Vemurafenib skin surveillance continues up to six months after stopping or until another anticancer therapy starts. Its other-cancer surveillance includes head-and-neck checks and chest CT, coordinated separately from melanoma-response imaging. A routine cobimetinib break is not evidence of toxicity recovery.

Can I use this without a phone?

Yes. The complete handbook, three-page summary and cut-out wallet card are printable. Ask the team to enter actual contact numbers and provide an updated paper plan when anything changes.

Try explaining the plan back: Can you explain your treatment setting, medicine schedule, next tests, the symptoms that need urgent advice and the number you would call?

The treatment this guide covers Only cobimetinib has the planned days 22–28 break. Vemurafenib continues. electrocardiogram (ECG) measures rhythm; an echocardiogram or other left ventricular ejection fraction (heart pumping measurement) (LVEF) test measures pumping function, and one does not replace the other. Radiation treatment needs a separate coordinated plan.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Cobimetinib + vemurafenib — advanced melanoma · support plan

Active medicines: Cobimetinib + Vemurafenib

Emetogenic risk: Cobimetinib minimal to low; vemurafenib minimal to low in NCCP 00373 v3; local policy

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- NCCP lists each component as minimal-to-low emetogenic risk; follow local and individual anti-sickness policy. Agree a diarrhoea, fluid-intake and nausea plan; contact the team if symptoms persist or you cannot drink. Check all prescribed, over-the-counter and herbal medicines for interactions before taking them. The seven-day cobimetinib break is planned; extra breaks or restarting doses require the team's instructions.
- Blood count, kidney/liver tests and lactate dehydrogenase (LDH) before each cycle. Current Cotellic product information adds creatine phosphokinase (CPK) and creatinine monthly, and LVEF after the first month then at least every three months until treatment stops. ECG is every four weeks for the first 12 weeks, then every 12 weeks and after dose changes. Arrange monthly skin checks under current vemurafenib product information. New visual symptoms need prompt eye assessment; symptoms can require extra tests between scheduled checks.
- Contact the team promptly for new blurred or lost vision, eye pain, breathlessness or ankle swelling, palpitations or fainting, severe muscle pain or weakness with dark urine, unusual bleeding, severe diarrhoea, a blistering rash or yellow skin or eyes.
- The cobimetinib source specifies two effective contraception methods during treatment and for at least three months afterwards; vemurafenib requires at least six months after its last dose. The combined plan therefore needs to cover the longer six-month vemurafenib interval and account for reduced hormonal-contraceptive effectiveness. Discuss fertility and breastfeeding before treatment.
- Only cobimetinib has the planned days 22–28 break. Vemurafenib continues. ECG measures rhythm; an echocardiogram or other LVEF test measures pumping function, and one does not replace the other. Radiation treatment needs a separate coordinated plan.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Eyes / retina

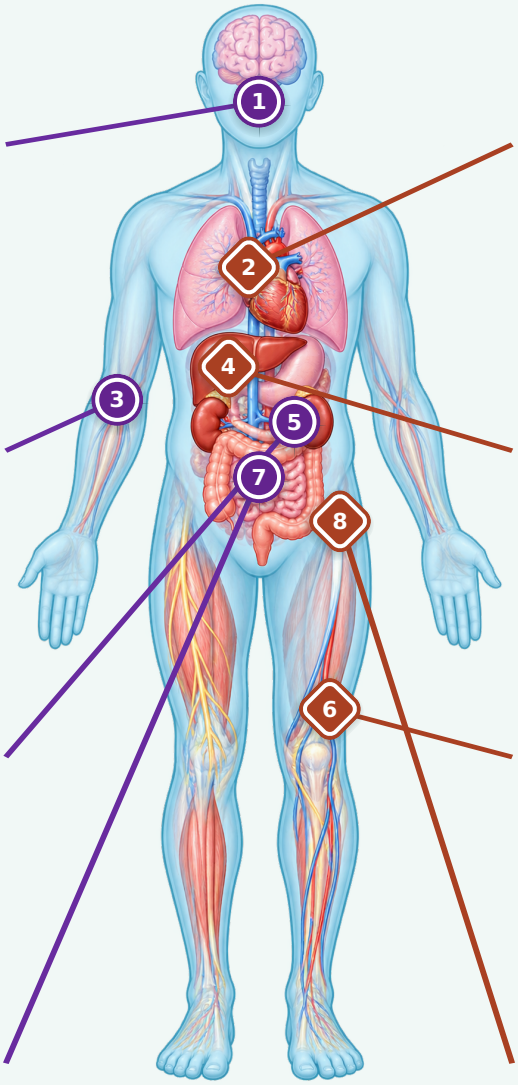
Cobimetinib + vemurafenib

New blurred vision, distortion, blind spots or eye pain can reflect retinal or inflammatory problems. Contact the team promptly rather than waiting for the next cycle.

2 Heart function / rhythm

Cobimetinib + vemurafenib

Cobimetinib can reduce heart pumping; vemurafenib can alter rhythm. Breathlessness, swollen ankles, palpitations or fainting need review.



3 Skin / sun

Cobimetinib + vemurafenib

Rash, photosensitivity and new skin lesions need protection and review. Severe blistering or peeling needs urgent help.

4 Liver

Cobimetinib + vemurafenib

Blood tests can detect liver injury before symptoms. Report jaundice, dark urine or upper-abdominal pain.

5 Kidneys / fluids

Cobimetinib + vemurafenib

Diarrhoea, vomiting or muscle damage can affect kidneys. Reduced urine or dark urine with muscle pain is urgent.

6 Muscles

Cobimetinib

Severe muscle pain, weakness or dark urine may signal muscle breakdown. The team may check muscle enzymes and kidneys.

7 Bowels

Cobimetinib + vemurafenib

Diarrhoea, nausea and abdominal discomfort can cause dehydration. Report persistent symptoms; severe pain or blood needs urgent assessment.

8 Bleeding

Cobimetinib

Unusual bruising, persistent nosebleeds, black or bloody stools, coughing blood or a severe new headache need prompt assessment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact the team promptly for new blurred or lost vision, eye pain, breathlessness or ankle swelling, palpitations or fainting, severe muscle pain or weakness with dark urine, unusual bleeding, severe diarrhoea, a blistering rash or yellow skin or eyes.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00373 v3 supportive-care section; current product information for medicine-specific precautions**
   https://healthservice.hse.ie/documents/6728/373_V3_Cobimetinib_and_Vemurafenib.pdf
2. **EMA Cotellic product information ↗**
   https://www.ema.europa.eu/en/documents/product-information/cotellic-epar-product-information_en.pdf
3. **EMA Zelboraf product information ↗**
   https://www.ema.europa.eu/en/documents/product-information/zelboraf-epar-product-information_en.pdf
4. **Ascierto et al. coBRIM, Lancet Oncology 2016**
   https://pubmed.ncbi.nlm.nih.gov/27480103/
5. **NCI: fatigue during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/fatigue
6. **NCI: melanoma — patient information**
   https://www.cancer.gov/types/skin/patient/melanoma-treatment-pdq
7. **NCI: melanoma treatment — clinical evidence**
   https://www.cancer.gov/types/skin/hp/melanoma-treatment-pdq
8. **NCI: eating and nutrition during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
9. **NCI: palliative care during cancer treatment**
   https://www.cancer.gov/about-cancer/advanced-cancer/care-choices/palliative-care-fact-sheet